# A/PROF CHAMARA BASNAYAKE
**MBBS, PhD, FRACP**
**Gastroenterologist & Endoscopist**
**PN 421926PK**

FOCUS GASTROENTEROLOGY


25 May 2026

Dr Kerryn Gijsbers
East Brunswick Medical Centre

Dear Dr Gijsbers,

**Re: Ms Janice Murphy**
**DOB: 01/03/1966 Phone: 0459 843 517**
**Address: 105 Barkly Street, FITZROY NORTH, VIC, 3068**
**Email: janice.m105@gmail.com**

**Summary**

- **Recent colonoscopy revealed no visual or histological evidence of inflammatory bowel disease or microscopic colitis.**
- **Normalisation of liver function tests and CRP, with an improving but still elevated ESR of 73.**
- **Plan to cease Creon and taper Salofalk granules.**
- **Strategy for rescue budesonide provided should symptoms of diarrhoea recur.**
- **Clinical features suggestive of Sjogren's syndrome noted; for symptomatic management.**
- **No action required by you.**

I had the pleasure of conducting a review with Janice Murphy following her colonoscopy. As you may recall, we were investigating significant gastrointestinal symptoms and an elevated faecal calprotectin.

Visually, the procedures did not reveal any evidence of Crohn's disease or ulcerative colitis. Despite the previously elevated calprotectin, the multiple biopsies I took have all returned as histologically normal, with no evidence of microscopic colitis. It is possible that the course of budesonide Janice completed prior to the procedure successfully treated the underlying inflammation, accounting for the normal findings.

Janice's biochemistry is very encouraging. Her liver function tests have completely normalised, and her CRP, which was previously 12, is now within the normal range. Her celiac serology is also moving in the right direction following her diagnosis in December. Her ESR has improved from 86 to 73; while this remains elevated, it appears to be trending downwards in the context of treating her recent inflammatory episode.

During our review, Janice mentioned symptoms of a dry mouth, dry eyes, and Raynaud's phenomenon. While I do not wish to over-diagnose her, these clinical features are highly suggestive of Sjogren's syndrome. Given that a formal diagnosis would not currently alter our management, I have suggested she use Biotene or artificial saliva as required. I have asked her to report any new symptoms such as skin tightness, chest pain, or exertional dyspnoea, which might necessitate further connective tissue disease evaluation.

**Impression and Plan**

My goal now is to rationalise Janice's medications. I have advised her to finish her current supply of Creon and then cease it. Regarding the Salofalk, I have asked her to reduce the dose to 1.5g (one sachet) daily for


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Endoscopy locations
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three months before stopping it entirely. It remains possible that Janice has an underlying ulcerative colitis that is currently well-controlled by the mesalazine.

If she experiences a significant symptomatic flare in the future, I have provided her with a rescue script for budesonide. She is to use the same tapering regime we utilised previously: 9mg for three weeks, 6mg for three weeks, and then 3mg for three weeks. If she finds she requires more than one such course a year, we will need to reconsider a long-term maintenance strategy.

Janice does not require a routine follow-up appointment with me at this stage, though she is welcome to return should her symptoms recur. Her surveillance gastroscopy for celiac disease is due in early 2026.

Yours sincerely,

**A/Prof Chamara Basnayake**
Gastroenterologist

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